



Histopathology

ICA-0-3

Specimen Comments

SPECIMEN

1. Right ureteric margin
2. Bladder
3. Right external iliac node
4. Right ureteric margin
5. Right obturator node
6. Right surgical resection margin of ureter
7. Left surgical ureteric resection margin

CQCF Carcinoma, Urothelial, NOS 8120/3

Path Carcinoma, transitional cell, NOS 8120/3

Site: Bladder, NOS C67.9

hw  
1/9/15

CLINICAL DETAILS

Radical cystectomy for G3 pT2 ureteric carcinoma. Tumour left bladder neck.

?Into prostate. ?Nodes right iliac.

MACROSCOPY

1. Piece of tissue 7mm diameter x 4mm length, which appears to have a central lesion.
2. Bladder and prostate measuring 100mm SI, transverse 55mm and AP 40mm with attached fatty tissue (182 x 120mm). On opening there is a bladder base tumour (left more than right), measuring 27mm AP x 32mm SI x 40mm transverse. It has a white homogeneous firm surface and appears to extend into the prostate. It is focally haemorrhagic. The tumour appears to extend to, but not breach the circumferential margin.
3. Fibrofatty tissue 35 x 25 x 12mm. No lymph nodes identified.
4. Tubular piece of tissue measuring 5 x 5 x 7mm.
5. Fibrofatty tissue which measures 30 x 35 x 10mm.
6. Tubular piece of tissue which measures 14 x 5 x 5mm.
7. Tubular piece of tissue which measures 15 x 4 x 4mm.

MICROSCOPY

1.

Frozen section report to

No evidence of carcinoma in-situ or invasive malignancy.

Paraffin section report

Paraffin sections show a largely morphologically normal ureter with a focal area having some nuclear crowding and variation in size sufficient for a diagnosis of focal dysplasia. There is no evidence of carcinoma in situ or urothelial neoplasia.

2. Sections of the macroscopically identified tumour confirm a poorly differentiated solid transitional cell carcinoma arising from dysplastic urothelium to invade through the muscle but lie free of the circumferential surgical margins. There are numerous foci of comedo-type necrosis. No unequivocal vascular invasion is noted. There is no perineural invasion. The tumour is seen to extend into prostatic ducts and in a pagetoid fashion and also invade the prostate. The prostatic circumferential margin is morphologically normal. The surviving prostatic glands are morphologically normal with no evidence of high grade PIN or evidence of microacinar adenocarcinoma.

The urethral surgical margin shows evidence of carcinoma in situ and also invasive carcinoma.

The background bladder shows morphologically normal urothelium with no evidence of dysplasia or carcinoma in situ.

The tumour is seen to extend very close to the seminal vesicle wall but does not involve the seminal vesicle per se. An accompanying giant cell reaction abutts the seminal vesicle tissue.

3. Right external iliac nodes. All the specimen was processed and contains mainly fibrovascular and adipose tissue only. Towards one end there is a focus of what appears to be a part of a lymph node with marked diathermy artefact which raises the possibility of invasion by urothelial carcinoma.

Immunohistochemistry has been requested and a supplementary report will be issued when results are available.

4. Right ureteric margin. This is ureteric tissue showing focal carcinoma in situ. There is no evidence of a papillary neoplasm.

5. Right obturator node; Five lymph nodes identified, two containing metastatic carcinoma with extension into extracapsular tissue.

6. Right surgical resection margin ureter; This is morphologically normal ureteric tissue with no evidence of carcinoma in situ or TCC.

7. Left surgical ureteric margin; This is ureteric tissue showing carcinoma in situ of the urothelium. There is no evidence of invasive malignancy.

SUMMARY: Radical cystectomy and lymphadenectomy; The appearances are those of a

poorly differentiated solid transitional cell carcinoma invading the prostate with regional node metastases. There is background carcinoma in situ and the urethral and left ureteric surgical margins show carcinoma in situ (pT4 pN2 pMx, Tis).

Ref:

Pathologists -

#### SUPPLEMENTARY REPORT

This report has been issued to give results of further scheduled tests.

Part 3 - Right externally iliac node; Immunohistochemistry confirms the area on interest to be composed of lymphocytes expressing CD45. There is no cytokeratin staining to suggest metastatic disease.

As such, the right external iliac node do not have any evidence of metastatic disease.

Please note the summary diagnosis remains unchanged.

Ref:

#### ORIGINAL REPORT

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Originally signed 1/9/14

| Criteria                       | Yes       | No                    |
|--------------------------------|-----------|-----------------------|
| Diagnosis Discrepancy          |           | ✓                     |
| Primary Tumor Site Discrepancy |           | ✓                     |
| HIPAA Discrepancy              |           | ✓                     |
| Prior Malignancy History       |           | ✓                     |
| Dual/Synchronous Primary Noted |           | ✓                     |
| Case is (circle):              | QUALIFIED | DISQUALIFIED          |
| Reviewer Initials              | SR        | Date Reviewed: 1/9/15 |